TSHR and IGF1R in Graves disease and thyroid eye disease: genetic evidence from blood expression

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Abstract

Objective: Thyroid eye disease (TED) can accompany Graves disease (GD), an autoimmune cause of hyperthyroidism. We compared genetic evidence linking blood *TSHR* and *IGF1R* expression with susceptibility.

Methods: We screened 2,544 druggable genes using Mendelian randomization (MR). Disease outcomes were Biobank Japan (BBJ) GD, UK Biobank (UKB) hyperthyroidism and overlapping FinnGen GD, Graves ophthalmopathy (GO) and strict GO endpoints. Colocalization tested 24 genes for a shared causal variant. Tissue eQTL and orbital RNA-sequencing (four TED, one control) were supplementary.

Results: Thirteen genes passed BBJ discovery ($P < 0.05/2{,}544$). Higher genetically proxied *TSHR* expression was associated with lower GD odds in BBJ (odds ratio [OR] per approximately one standard deviation 0.12, 95% confidence interval [CI] 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$), concordantly across all five datasets. Shared-variant probability exceeded 0.80 in FinnGen GO and, prior-sensitively, BBJ and strict GO, but UKB (0.226) and FinnGen GD (0.264) favored distinct variants. *IGF1R* ORs exceeded 1 but reached $P < 0.05$ only in BBJ (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB ($P = 0.0117$); the most precise estimate, FinnGen GD, was OR 1.07 (95% CI 0.86–1.33). Single-variant tissue estimates added no independent evidence. Mean orbital *TSHR* and *IGF1R* transcript levels exceeded the control.

Conclusions: Association evidence was stronger for *TSHR* than *IGF1R*, but absent colocalization in UKB and FinnGen GD leaves expression-mediated causality unproven. Population-controlled data cannot isolate TED susceptibility within GD; weaker *IGF1R* evidence does not argue against IGF-1R–targeted treatment.

Keywords: Graves disease; thyroid eye disease; *TSHR*; *IGF1R*; Mendelian randomization; colocalization

Introduction

Thyroid eye disease (TED), or Graves ophthalmopathy, can cause proptosis, diplopia and visual loss in patients with Graves disease (GD) [1, 2]; a meta-analysis estimated that it affects about 40% of them [3]. In GD, autoantibodies that stimulate the thyroid-stimulating hormone receptor (TSHR) cause hyperthyroidism [2]. Common *TSHR* variants are established GD risk factors in European and East Asian populations [4, 5], but no TED-specific risk variant is known [6]. In orbital fibroblasts, TSHR forms a signaling complex with the insulin-like growth factor 1 receptor (IGF-1R) [7, 8], and the IGF-1R–blocking antibody teprotumumab reduced proptosis in randomized trials in active TED [9, 10]. Genetically supported drug targets succeed more often, but most approved drug mechanisms lack such support [11, 12], so an effective target need not be a susceptibility gene. Whether inherited variation in *IGF1R* expression, like *TSHR* variation, is associated with GD or TED is unclear.

Mendelian randomization (MR) treats inherited variants as a natural experiment: because alleles are allocated at conception, comparing people whose variants raise or lower a gene's expression is less prone to confounding and reverse causation than other observational comparisons, provided the variants affect disease only through that expression [13]. Their effects are lifelong, unlike a drug's, and can differ between tissues [14]. Here, genetically proxied expression means blood expression predicted from inherited variants rather than measured directly, and the outcomes are diagnoses, not treatment responses. MR reports odds ratios (ORs) with 95% confidence intervals (CIs). Colocalization asks whether expression and disease signals in a region share one causal variant and reports probabilities, not *P* values [15]. Because nearby variants are often inherited together, an MR association can be highly significant even when the two signals come from distinct variants [16, 17].

In FinnGen release 10, Ji and colleagues linked higher genetically proxied blood *TSHR* expression to lower odds of GD and strict ophthalmopathy [18]. Using release 12 endpoints also analyzed here, Zhang and colleagues reported shared-variant support for *TSHR* with the broader ophthalmopathy endpoint but weak support with GD [19]. A proteome-wide MR study of GD and ophthalmopathy prioritized other proteins [20]. None of these studies compared *IGF1R* with *TSHR*, and all relied mainly on FinnGen.

Within an MR screen of potentially druggable genes [21], we therefore compared genetic evidence linking blood *TSHR* and *IGF1R* expression with GD in Biobank Japan (BBJ; discovery) and FinnGen, hyperthyroidism in UK Biobank (UKB) and two ophthalmopathy endpoints in FinnGen. *CTLA4*, an established autoimmune susceptibility gene, served as comparator [5, 22]. The three FinnGen endpoints overlap and are not independent replications. We examined colocalization and sensitivity to single variants, allele frequencies and colocalization assumptions; FinnGen GD, strict ophthalmopathy, tissue-specific and several sensitivity analyses were post hoc. Because the ophthalmopathy endpoints used population controls, they cannot estimate TED risk among patients with GD, and no analysis predicts treatment response. Orbital RNA-sequencing data (four TED samples, one control) are descriptive only.

Methods

Study design and disease datasets

This two-sample MR study used public association summary statistics and followed STROBE-MR reporting guidance [23]. Exposure data were whole-blood expression quantitative trait locus (eQTL; a variant associated with gene expression) statistics from predominantly European eQTLGen participants [24]. The original outcomes were BBJ GD, UKB self-reported hyperthyroidism or thyrotoxicosis, and Graves ophthalmopathy in FinnGen release 12 (FinnGen ophthalmopathy) [25, 26, 27]. We later added two FinnGen

endpoints: E4_GRAVES_STRICT (FinnGen GD) and E4_GRAVES_OPHT_STRICT (strict ophthalmopathy). Table 1 gives case and control counts. UKB ancestry was not established as exclusively European.

BBJ was the discovery dataset, FinnGen GD a same-disease comparison, UKB a broader-phenotype comparison, FinnGen ophthalmopathy a TED-enriched comparison and strict ophthalmopathy a definition sensitivity analysis. The three FinnGen endpoints share participants and were neither treated as independent replications nor meta-analyzed. Their controls were all other participants, not GD patients without TED. The FinnGen extensions, expanded candidate assessment and instrument diagnostics were post hoc; the study was not prospectively registered. Analyses used R 4.3.3, TwoSampleMR 0.7.4, coloc 5.2.3 and PLINK 1.9, with independent numerical verification in Python.

Expression instruments and MR

For 4,462 potentially druggable genes [21], instruments (variants used to represent a gene's expression) were blood eQTLs with $P < 5\times10^{-8}$ within 1 Mb of the gene (cis-eQTLs). PLINK clumping kept only uncorrelated variants (linkage disequilibrium [LD] $r^2 < 0.001$ within 10 Mb, in 503 European participants from 1000 Genomes Phase 3) [28, 29]. This gave 6,135 single-nucleotide polymorphism (SNP)–gene pairs for 2,544 genes (minimum F statistic 29.7; $F > 10$ is the usual threshold for adequate instrument strength), and the extended major histocompatibility complex region was not excluded. Variant effects on expression, in standard deviation units, were derived from eQTLGen Z statistics, sample sizes and 1000 Genomes European allele frequencies [30]. TwoSampleMR aligned exposure and disease effects to the same allele, excluding unavailable and strand-ambiguous (palindromic) variants. UKB linear-model estimates were rescaled using the case proportion to approximate the log-odds scale of BBJ and FinnGen (Supplementary Methods).

For each gene and dataset, MR used the Wald ratio (disease effect divided by expression effect) for one instrument and, for several, multiplicative random-effects inverse-variance weighting (IVW), a precision-weighted average [31]. Estimates are ORs per approximately one standard deviation higher genetically proxied blood expression, with 95% CIs and two-sided *P* values. The BBJ screen threshold was Bonferroni-corrected for all 2,544 instrumented genes, estimable or not ($P < 0.05/2{,}544 = 1.965\times10^{-5}$); it was applied descriptively to the other datasets and in an exploratory full FinnGen GD screen. The 13 BBJ discoveries were assessed at a focused FinnGen GD threshold ($P < 0.05/13$ and the same direction). Nominal (uncorrected) $P < 0.05$ was reported descriptively. ORs detectable with 80% power were approximated from observed standard errors (Supplementary Methods).

MR assumes that instruments predict expression, are unrelated to confounders of expression and disease, and affect disease only through that gene's expression.

For estimates with several instruments, Cochran's Q test assessed heterogeneity among instruments and, for selected genes, post hoc omission of each instrument in turn (leave-one-out) assessed dependence on single variants. MR-Egger regression, whose intercept tests whether instruments affect disease through other pathways (directional pleiotropy), and weighted-median and weighted-mode estimators [32, 33] were applied only in the original three datasets (Supplementary Dataset, MR_estimators_original). Analyses were repeated with exposure effects reconstructed from eQTLGen cohort allele frequencies; for *C4B* and *HLA-G*, whose FinnGen GD screen result changed, instrument weights and omissions were compared between frequency sources (Supplementary Methods).

In a post hoc audit, rs179252 and rs179247 (the variant examined in published thymic *TSHR* studies) were re-extracted from the original expression and disease files. Disease effects were aligned to the blood expression-increasing allele to give per-allele ORs and recalculate the

MR estimates, and the correlation between the two variants' G-allele counts was calculated in European and East Asian 1000 Genomes genotypes. The audit checked allele direction, not thymic mediation (Supplementary Dataset, TSHR_allele_audit).

Colocalization

MR associations are the main findings, and colocalization was used to interpret them; the original analysis reported both in parallel, and this emphasis was adopted post hoc without prospective registration. We used coloc.abf, which assumes at most one causal variant per trait, with all available eQTLGen cis-eQTL statistics for each gene, significant or not, matched to each disease dataset [15]. It was applied to 24 genes: the 13 BBJ discoveries, *IGF1R*, the nine additional FinnGen GD screen discoveries and *HLA-G*, added because its MR result changed with the allele-frequency source. Posterior probabilities (PP.H0–PP.H4) were estimated for five hypotheses: neither trait associated (H0), disease only (H1), expression only (H2), distinct causal variants (H3) and one shared causal variant (H4). Prior probabilities that a variant is associated with disease (p1), expression (p2) or both (p12) were 10^{-4}, 10^{-4} and 10^{-5}, with p12 also set to 5×10^{-6} and 10^{-6} [34]. PP.H4 ≥ 0.80 was a descriptive reporting threshold.

For *TSHR*, colocalization was repeated in narrower regions and in a variant set common to all five datasets, and we checked whether the matched variants included each dataset's strongest regional disease variant (Supplementary Methods).

Tissue-specific extension

In a post hoc extension, we assessed *TSHR* and *IGF1R* in a fixed panel of five Genotype-Tissue Expression (GTEx) v8 tissues: thyroid, cultured skin fibroblasts, subcutaneous adipose, visceral adipose and whole blood [35]. The plan was fixed after the blood results were known but before tissue results were retrieved. GTEx donors were predominantly

European. Instruments required cis-eQTL $P < 5\times10^{-8}$, F > 10 and European-reference LD $r^2 <$ 0.001. Ten planned gene–tissue comparisons against FinnGen GD formed the primary family ($P < 0.005$); 40 comparisons against the other four datasets formed a secondary family ($P <$ 0.00125). Unestimable combinations remained in the correction denominators. Effects used GTEx slopes and standard errors for inverse-normal-transformed expression. Tissue colocalization was not performed because complete regional statistics were not obtained.

Descriptive orbital expression

In-house RNA sequencing of orbital adipose tissue from four patients with inactive TED and of an upper-eyelid nasal fat pad from one control without thyroid disease was descriptive only (Supplementary Methods). Transcript abundance was displayed without significance testing (Figure S1) and was not used to select genes or validate the genetic findings.

Results

Discovery across druggable genes

Of the 2,544 instrumented genes, 2,234 were estimable in BBJ, 2,505 in UKB and 2,480 in each FinnGen endpoint. Thirteen genes passed the BBJ screen threshold ($P < 0.05/2{,}544$), including *TSHR* and *CTLA4* but not *IGF1R* (Figure 1). At this threshold, 14.6% and 35.6% of estimable BBJ genes had 80% power to detect ORs of 1.5 and 2.0, respectively (Supplementary Dataset, Detection_original).

TSHR and IGF1R across disease datasets

Higher genetically proxied blood *TSHR* expression was associated with lower odds of GD in BBJ (OR 0.12, 95% CI 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$). ORs were also below 1 for UKB hyperthyroidism and both ophthalmopathy endpoints, and all estimates except strict ophthalmopathy passed the screen threshold (Table 2; Figure 2). Each estimate used the single instrument rs179252. Its

expression-increasing G allele had per-allele disease ORs of 0.77–0.82 across datasets and was correlated with rs179247-G (European r = 0.978; East Asian r = 0.996), which also increased blood *TSHR* expression. Allele-aligned recalculation reproduced the MR estimates (Supplementary Dataset, TSHR_allele_audit).

All five *IGF1R* ORs exceeded 1, and none passed the screen threshold. Nominal associations (uncorrected $P < 0.05$) were found only in BBJ GD (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB hyperthyroidism (OR 1.35, 95% CI 1.07–1.70; $P = 0.0117$). The most precise estimate, FinnGen GD, was OR 1.07 (95% CI 0.86–1.33; $P = 0.539$); this comparison had 80% power to detect an OR of 1.36 at $P < 0.05$ (Supplementary Methods). Weighted-median ORs were 1.56 ($P = 0.0288$) in BBJ, 1.42 ($P = 0.00244$) in UKB and 1.46 ($P = 0.124$) in FinnGen ophthalmopathy. In these three datasets, MR-Egger and weighted-mode estimates had $P \geq 0.32$, and MR-Egger intercept P values were 0.66, 0.56 and 0.43, respectively (Supplementary Dataset, MR_estimators_original).

Other associated genes

Four genes passed the screen threshold in both GD datasets: *TSHR*, *CTLA4*, *HLA-DQA2* and *TUBB* (Table 3). *CTLA4* was associated with lower odds, and *HLA-DQA2* and *TUBB*, both in the major histocompatibility complex (MHC) region, with higher odds. *CTLA4* and *TUBB* had direction-consistent nominal associations in all five datasets; *HLA-DQA2* did in four, but its two-instrument UKB estimate was non-significant and below 1 (Supplementary Dataset, MR_primary). At the focused FinnGen GD threshold ($P < 0.05/13 = 0.00385$), seven of the 13 BBJ discoveries had direction-consistent associations: these four genes plus *C4A*, *TNFSF14* and *IFNGR1*. *HLA-A*, which had the smallest BBJ P value, had a non-significant FinnGen GD estimate.

The exploratory full FinnGen GD screen identified 13 genes, nine of them outside the BBJ discovery set (Table S1). Seven of the nine were instrumented in the extended MHC region, and two pairs each shared one instrument.

Instrument and frequency sensitivity

Omitting rs13030124 left non-significant *CTLA4* estimates in UKB and all three FinnGen endpoints (P = 0.323–0.972; Figure S2). In BBJ and UKB, where *IGF1R* had four instruments, omitting each in turn gave $P > 0.05$ in two of the four estimates; omitting rs2654980 in UKB also lowered the OR from 1.35 to 1.16 (Figure S2). Among the BBJ discoveries, Cochran's Q test in FinnGen GD indicated heterogeneity among instruments for *C4A* ($P = 1.27\times10^{-12}$), *TUBB* ($P = 3.67\times10^{-5}$), *HLA-A* (P = 0.0387) and *CTLA4* (Table 2). Omitting rs149543464 (*TUBB*) or rs1269852 (*C4A*) raised the FinnGen GD P value to 0.0262 and 0.0471, respectively (Supplementary Dataset, LOO_new_fixed14).

With eQTLGen cohort allele frequencies, all 13 BBJ discoveries still passed the screen threshold, *TSHR P* values were unchanged and no *IGF1R P* value crossed 0.05 (Supplementary Dataset, MR_original_freq and MR_new_freq). Screen classification changed only in FinnGen GD: *C4B* no longer passed, and *HLA-G* newly passed; the IVW weight of rs150254595 in its estimate rose from 21.3% to 58.9% (Table S2; Supplementary Dataset, Instrument_weights). Omitting this variant left non-significant *HLA-G* estimates under either frequency source (Supplementary Dataset, LOO_frequency).

Colocalization

TSHR PP.H4 (probability of one shared causal variant) was 0.951 in BBJ, 0.986 in FinnGen ophthalmopathy and 0.891 in strict ophthalmopathy, but 0.226 in UKB and 0.264 in FinnGen GD, where PP.H3 (distinct variants) was highest (Table 2; Figures 2 and 3). The UKB and FinnGen GD values were unchanged in narrower regions and in a variant set common to all

five datasets, and the matched variants included the strongest regional disease variant in each dataset (Table S3). Lowering the prior probability of a shared variant (p12) from 10^{-5} to 10^{-6} gave PP.H4 of 0.875 in FinnGen ophthalmopathy, 0.661 in BBJ and 0.451 in strict ophthalmopathy.

IGF1R PP.H4 ranged from 0.010 to 0.404 (UKB); in the other four datasets, PP.H2 (expression signal only) was highest (0.571–0.731). *CTLA4* PP.H4 was ≥ 0.80 in UKB and both ophthalmopathy endpoints but not in BBJ or FinnGen GD. None of the 24 genes reached PP.H4 ≥ 0.80 in FinnGen GD, and *HLA-DQA2* and *TUBB* had PP.H4 < 0.001 in every dataset. Apart from *TSHR* and *CTLA4*, only *TNFSF14* and *IFNGR1* reached PP.H4 ≥ 0.80 in any dataset, both in BBJ only (Figure 3).

Tissue-specific extension

Four of the ten gene–tissue combinations had a qualifying instrument, each a single SNP: *TSHR* in subcutaneous and visceral adipose and *IGF1R* in thyroid and cultured fibroblasts. Thus, 20 of the 50 planned comparisons were estimable and 30 were not (Table S4). All ten adipose *TSHR* ORs were below 1, and eight, including both primary FinnGen GD comparisons, passed the multiple-testing threshold of their test family. The adipose instruments, rs12101255 (subcutaneous) and rs74064796 (visceral), lie 50 bp apart in *TSHR* intron 1, about 15 kb from rs179252 (European r^2 0.46 for rs12101255), and both are blood *TSHR* eQTLs (eQTLGen $P = 4.1\times10^{-34}$ and 6.5×10^{-34}). With one instrument, each tissue *P* value equals that variant's disease-association z-test *P* value, so the adipose estimates re-express the regional *TSHR* association. No thyroid or cultured-fibroblast *IGF1R* estimate reached $P < 0.05$, and directions were inconsistent across datasets.

Descriptive orbital expression

Mean orbital *TSHR*, *IGF1R* and *CTLA4* transcript levels in four patients with inactive TED exceeded those in the single control, whose sample came from a different fat depot; no significance testing was performed (Figure S1).

Discussion

Higher genetically proxied blood *TSHR* expression was associated with lower GD odds in BBJ and FinnGen GD, with concordant directions for UKB hyperthyroidism and both ophthalmopathy endpoints. All five *IGF1R* ORs exceeded 1, but only BBJ and UKB were nominally significant, none passed the screen threshold, and FinnGen GD, the most precise, lay close to the null. Under the primary prior, colocalization favored a shared *TSHR* variant except in UKB and FinnGen GD. Significance for one gene but not the other is not a formal test of different effects.

Ji and colleagues linked blood *TSHR* expression to GD and strict ophthalmopathy in an earlier FinnGen release [18], and integrative work also investigated *TSHR* [19]. We add an *IGF1R* comparison, GD in two populations and a druggable-gene screen. Both studies used eQTLGen and FinnGen, so agreement is not fully independent. Ji and colleagues accepted PP.H4 above 0.50 [18], whereas we used PP.H4 $\geq$ 0.80 with p12 = 10^{-5}; with different methods and releases, the labels are not directly comparable and differences are not contradictions.

The multi-instrument FinnGen and UKB *CTLA4* estimates relied heavily on rs13030124; with that caveat, its direction-consistent associations across all five datasets support it as an autoimmune comparator [22]. *HLA-A*, the strongest BBJ association, was non-significant in FinnGen GD, so the smallest discovery *P* value need not transfer best. *HLA-DQA2* was discordant only in its one two-instrument estimate, UKB, whose instruments were strongly

heterogeneous (Supplementary Dataset, MR_estimators_original). With shared instruments, MHC linkage, single-variant dependence of the FinnGen GD *TUBB* and *C4A* estimates, one regional association behind the three chromosome 16p11.2 genes, and no shared-variant support for *HLA-DQA2* or *TUBB*, these associations cannot each be counted as an independent causal target.

The lower-odds direction need not contradict stimulating-antibody pathogenesis: blood transcript abundance, tissue expression and receptor activation are different exposures with tissue-dependent regulatory effects [35]. The expression-increasing rs179252-G allele is linked to rs179247-G, the GD-protective allele associated with greater intrathymic *TSHR* transcription [36, 37]; this fits central tolerance, though the thymic step was untested and neither variant is a proven causal regulator. Each rs179252-G allele raises blood expression by 0.105 standard deviation, and its per-allele disease ORs were 0.80 (BBJ), 0.77 (UKB), 0.78 (FinnGen ophthalmopathy), 0.77 (FinnGen GD) and 0.82 (strict ophthalmopathy); rescaled to one standard deviation, these moderate effects give the reported odds ratios, which describe lifelong inherited differences, not raised *TSHR* expression or TSHR-directed treatment.

Very small *TSHR* MR *P* values in UKB and FinnGen GD coexisted with distinct-variant patterns, as for *CTLA4* in BBJ and FinnGen GD, so association strength and shared-variant evidence are not interchangeable: a non-zero MR estimate can arise from distinct but correlated variants [16]. These two datasets had the most cases, and their strongest regional disease variant was more strongly associated than rs179252 (Table S3); regional checks excluded a missing-peak explanation but did not identify the causal variant. Support in BBJ and strict ophthalmopathy fell below 0.80 only at the lowest prior tested (Figure 3B). The adipose instruments are themselves blood *TSHR* eQTLs, so their estimates re-express the

regional association and cannot resolve the pattern or establish an adipose mechanism. These data establish association, not expression-mediated causality.

In mice, the shed TSHR A-subunit induced more thyroid-stimulating autoimmunity than a minimally cleaving receptor [38]; plasmid immunization reproduced orbital adipogenesis, fibrosis and muscle pathology [39], and orbital fibroblasts from immunized mice showed enhanced inflammatory and hyaluronan responses to TNFα [40]. Ophthalmopathy also occurs in euthyroid and hypothyroid patients [41]. Our instruments measure neither receptor cleavage nor antibody activity. Because most patients with TED have GD, the controls included people with GD but no recorded eye disease, and the ophthalmopathy per-allele ORs resembled those for GD (Table S3), these estimates may mostly reflect GD susceptibility; their higher PP.H4 values, from the two smallest, weakest-signal datasets, do not establish a TED-specific effect.

For *IGF1R*, blood expression-related evidence was limited: colocalization mostly favored an expression-only signal, consistent with weak regional disease signals, and some single-variant omissions removed nominal significance, though most left ORs close to the full estimates (Figure S2). The near-null FinnGen GD estimate argues against large, but not modest, effects; non-significance is not evidence of no inherited role. Single-instrument tissue estimates and absent orbital eQTL data leave tissue effects open. Weaker genetic evidence for *IGF1R* does not argue against IGF-1R blockade: teprotumumab reduced proptosis in a randomized trial in active TED [10] and in long-duration, low-activity TED [42], and TSHR–IGF-1R crosstalk [8] concerns receptor signaling, not inherited susceptibility.

These results inform research rather than treatment selection, leaving unresolved the tissue mediating the *TSHR* association and any inherited *IGF1R* contribution, which independent

instruments, thymic and orbital regulatory data and multi-signal colocalization could address. Orbital susceptibility requires comparing GD patients with and without TED, accounting for follow-up and selection into GD.

Limitations

BBJ analyses assumed that predominantly European eQTLGen effects and LD transfer to East Asians, in both the MR and colocalization. UKB self-reported hyperthyroidism is broader than GD and was not established as exclusively European. The three overlapping, population-controlled FinnGen endpoints are not independent replications, do not compare GD patients with and without TED, and cannot separate cohort from phenotype differences; participant overlap with eQTLGen was unquantified. Blood is neither thymus nor orbit: the predominantly European GTEx extension had four single-instrument gene–tissue combinations, reused the disease data and lacked colocalization, and adipose and cultured fibroblasts are not orbital tissue, with 30 unestimable comparisons that are not negative findings. The orbital series (inactive TED; one control from a different fat depot) is descriptive only.

The FinnGen extensions, candidate expansion and colocalization emphasis were post hoc, and only the screen, focused and tissue-family thresholds address specified test families. The single *TSHR* instrument precludes pleiotropy and omission tests and cannot exclude neighboring or correlated variants; other genes have few or heterogeneous instruments, MR-Egger has little power with three or four *IGF1R* instruments, and frequency substitution and omission reused the same data. Under the single-causal-variant model, a low PP.H4 can reflect model mismatch or complex MHC LD; multi-signal colocalization [43], directionality and formal between-phenotype or between-gene tests were not performed, and only 24 genes were assessed. At the screen threshold, most BBJ genes lacked 80% power for an OR of 1.5,

so a non-significant MR result, a PP.H4 below 0.80 or an unestimable comparison does not establish absence of an effect.

Conclusions

Higher genetically proxied blood *TSHR* expression was associated with lower odds of GD, hyperthyroidism and ophthalmopathy in every dataset. *IGF1R* odds ratios exceeded 1 in every dataset but nominally significant only in BBJ and UKB, and the most precise estimate, FinnGen GD, lay close to the null. Colocalization supported a shared *TSHR* variant in FinnGen ophthalmopathy and, except at the lowest prior tested, in BBJ and strict ophthalmopathy, but favored distinct variants in UKB and FinnGen GD. These analyses addressed disease susceptibility, not treatment response, and the *IGF1R* evidence does not argue against IGF-1R blockade. The ophthalmopathy analyses used population controls and did not estimate TED susceptibility among patients with GD.

Declarations

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Conflict of interest. The authors declare that they have no conflict of interest.

Ethics approval. This study used publicly available, de-identified summary statistics and an institutionally approved orbital tissue dataset. The in-house orbital transcriptomic component was approved by the Institutional Review Board of Pusan National University Hospital (approval number 2104-018-102) and was conducted in accordance with the Declaration of Helsinki.

Informed consent. Written informed consent was obtained from all individual participants included in the in-house orbital tissue study.

Data availability. Instruments, primary MR and colocalization results, and sensitivity results are supplied as Supplementary Dataset. Analysis code is available from the corresponding author on reasonable request. Public summary statistics analyzed in this study are available from their original repositories: blood *cis*-eQTL data from the eQTLGen Consortium, tissue eQTL data from GTEx v8, the Biobank Japan Graves disease and UK Biobank hyperthyroidism genome-wide association statistics through the GWAS Catalog, and the FinnGen Release 12 GD and ophthalmopathy statistics from FinnGen. The in-house orbital RNA-seq data are available from the corresponding author on reasonable request, subject to institutional and ethical restrictions.

Author contributions. J.P. and M.-S.K. contributed equally to this work; both performed the analyses and drafted the manuscript. J.P. conceived and designed the study. M.-S.K. contributed to data collection and interpretation. S.-W.Y. supervised the study and revised the manuscript.

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References

1. Bahn RS. Graves' ophthalmopathy. N Engl J Med. 2010;362:726-738. doi:10.1056/nejmra0905750.
2. Smith TJ, Hegedüs L. Graves' Disease. N Engl J Med. 2016;375:1552-1565. doi:10.1056/nejmra1510030.
3. Chin YH, Ng CH, Lee MH, Koh JWH, Kiew J, Yang SP, Sundar G, Khoo CM. Prevalence of thyroid eye disease in Graves' disease: A meta-analysis and systematic review. Clin Endocrinol (Oxf). 2020;93:363-374. doi:10.1111/cen.14296.
4. Brand OJ, Barrett JC, Simmonds MJ, Newby PR, McCabe CJ, Bruce CK, Kysela B, Carr-Smith JD, Brix T, Hunt PJ, et al. Association of the thyroid stimulating hormone receptor gene (TSHR) with Graves' disease. Hum Mol Genet. 2009;18:1704-1713. doi:10.1093/hmg/ddp087.
5. Chu X, Pan CM, Zhao SX, Liang J, Gao GQ, Zhang XM, Yuan GY, Li CG, Xue LQ, Shen M, et al. A genome-wide association study identifies two new risk loci for Graves' disease. Nat Genet. 2011;43:897-901. doi:10.1038/ng.898.
6. Marinò M, Rotondo Dottore G, Menconi F, Comi S, Cosentino G, Rocchi R, Latrofa F, Figus M, Santini F. Role of genetics and epigenetics in Graves' orbitopathy. Eur Thyroid J. 2024;13:e240179. doi:10.1530/ETJ-24-0179.
7. Tsui S, Naik V, Hoa N, Hwang CJ, Afifiyan NF, Sinha Hikim A, Gianoukakis AG, Douglas RS, Smith TJ. Evidence for an association between thyroid-stimulating hormone and insulin-like growth factor 1 receptors: a tale of two antigens implicated in Graves' disease. J Immunol. 2008;181:4397-4405. doi:10.4049/jimmunol.181.6.4397.
8. Krieger CC, Boutin A, Jang D, Morgan SJ, Banga JP, Kahaly GJ, Klubo-Gwiezdzinska J, Neumann S, Gershengorn MC. Arrestin-β-1 Physically Scaffolds

TSH and IGF1 Receptors to Enable Crosstalk. Endocrinology. 2019;160:1468-1479. doi:10.1210/en.2019-00055.

9. Smith TJ, Kahaly GJ, Ezra DG, Fleming JC, Dailey RA, Tang RA, Harris GJ, Antonelli A, Salvi M, Goldberg RA, et al. Teprotumumab for Thyroid-Associated Ophthalmopathy. N Engl J Med. 2017;376:1748-1761. doi:10.1056/NEJMoa1614949.
10. Douglas RS, Kahaly GJ, Patel A, Sile S, Thompson EHZ, Perdok R, Fleming JC, Fowler BT, Marcocci C, Marinò M, et al. Teprotumumab for the Treatment of Active Thyroid Eye Disease. N Engl J Med. 2020;382:341-352. doi:10.1056/nejmoa1910434.
11. Nelson MR, Tipney H, Painter JL, Shen J, Nicoletti P, Shen Y, Floratos A, Sham PC, Li MJ, Wang J, et al. The support of human genetic evidence for approved drug indications. Nat Genet. 2015;47:856-860. doi:10.1038/ng.3314.
12. Minikel EV, Painter JL, Dong CC, Nelson MR. Refining the impact of genetic evidence on clinical success. Nature. 2024;629:624-629. doi:10.1038/s41586-024-07316-0.
13. Sanderson E, Glymour MM, Holmes MV, Kang H, Morrison J, Munafò MR, Palmer T, Schooling CM, Wallace C, Zhao Q, et al. Mendelian randomization. Nat Rev Methods Primers. 2022;2:6. doi:10.1038/s43586-021-00092-5.
14. Gill D, Georgakis MK, Walker VM, Schmidt AF, Gkatzionis A, Freitag DF, Finan C, Hingorani AD, Howson JMM, Burgess S, et al. Mendelian randomization for studying the effects of perturbing drug targets. Wellcome Open Res. 2021;6:16. doi:10.12688/wellcomeopenres.16544.2.
15. Giambartolomei C, Vukcevic D, Schadt EE, Franke L, Hingorani AD, Wallace C, Plagnol V. Bayesian test for colocalisation between pairs of genetic association

studies using summary statistics. PLoS Genet. 2014;10:e1004383. doi:10.1371/journal.pgen.1004383.

16. Zuber V, Grinberg NF, Gill D, Manipur I, Slob EAW, Patel A, Wallace C, Burgess S. Combining evidence from Mendelian randomization and colocalization: Review and comparison of approaches. Am J Hum Genet. 2022;109:767-782. doi:10.1016/j.ajhg.2022.04.001.
17. Zheng J, Haberland V, Baird D, Walker V, Haycock PC, Hurle MR, Gutteridge A, Erola P, Liu Y, Luo S, et al. Phenome-wide Mendelian randomization mapping the influence of the plasma proteome on complex diseases. Nat Genet. 2020;52:1122-1131. doi:10.1038/s41588-020-0682-6.
18. Ji Q, Xu H, Chen H, Chen X, Wang S, Zou J. Pathogenic genes associated with immune-related genes in graves' disease: a multi-omics Mendelian randomization analysis. Sci Rep. 2025;15:37875. doi:10.1038/s41598-025-21754-4.
19. Zhang H, Jiang S, Zhu T, Liu Y, Li J, Fang S, Li Y, Sun J, He X, Tong C, et al. Thyroid-stimulating hormone receptor mediates peripheral-central neuroimmune crosstalk in autoimmune thyroid diseases. BMC Med. 2026;24:404. doi:10.1186/s12916-026-04957-y.
20. Ke C, Yu Y, Li J, Yu Y, Sun Y, Wang Y, Wang B, Lu Y, Tang M, Wang N, et al. Genetic and Plasma Proteomic Approaches to Identify Therapeutic Targets for Graves' Disease and Graves' Ophthalmopathy. Immunotargets Ther. 2025;14:87-98. doi:10.2147/ITT.S494692.
21. Finan C, Gaulton A, Kruger FA, Lumbers RT, Shah T, Engmann J, Galver L, Kelley R, Karlsson A, Santos R, et al. The druggable genome and support for target identification and validation in drug development. Sci Transl Med. 2017;9:eaag1166. doi:10.1126/scitranslmed.aag1166.

22. Ueda H, Howson JM, Esposito L, Heward J, Snook H, Chamberlain G, Rainbow DB, Hunter KM, Smith AN, Di Genova G, et al. Association of the T-cell regulatory gene CTLA4 with susceptibility to autoimmune disease. Nature. 2003;423:506-511. doi:10.1038/nature01621.
23. Skrivankova VW, Richmond RC, Woolf BAR, Yarmolinsky J, Davies NM, Swanson SA, VanderWeele TJ, Higgins JPT, Timpson NJ, Dimou N, et al. Strengthening the Reporting of Observational Studies in Epidemiology Using Mendelian Randomization: The STROBE-MR Statement. JAMA. 2021;326:1614-1621. doi:10.1001/jama.2021.18236.
24. Võsa U, Claringbould A, Westra HJ, Bonder MJ, Deelen P, Zeng B, Kirsten H, Saha A, Kreuzhuber R, Yazar S, et al. Large-scale cis- and trans-eQTL analyses identify thousands of genetic loci and polygenic scores that regulate blood gene expression. Nat Genet. 2021;53:1300-1310. doi:10.1038/s41588-021-00913-z.
25. Sakaue S, Kanai M, Tanigawa Y, Karjalainen J, Kurki M, Koshiba S, Narita A, Konuma T, Yamamoto K, Akiyama M, et al. A cross-population atlas of genetic associations for 220 human phenotypes. Nat Genet. 2021;53:1415-1424. doi:10.1038/s41588-021-00931-x.
26. Dönertaş HM, Fabian DK, Valenzuela MF, Partridge L, Thornton JM. Common genetic associations between age-related diseases. Nat Aging. 2021;1:400-412. doi:10.1038/s43587-021-00051-5.
27. Kurki MI, Karjalainen J, Palta P, Sipilä TP, Kristiansson K, Donner KM, Reeve MP, Laivuori H, Aavikko M, Kaunisto MA, et al. FinnGen provides genetic insights from a well-phenotyped isolated population. Nature. 2023;613:508-518. doi:10.1038/s41586-022-05473-8.

488 28. Chang CC, Chow CC, Tellier LC, Vattikuti S, Purcell SM, Lee JJ. Second-generation
489 PLINK: rising to the challenge of larger and richer datasets. Gigascience. 2015;4:7.
490 doi:10.1186/s13742-015-0047-8.
491 29. 1000 Genomes Project Consortium, Auton A, Brooks LD, Durbin RM, Garrison EP,
492 Kang HM, Korbel JO, Marchini JL, McCarthy S, McVean GA, et al. A global
493 reference for human genetic variation. Nature. 2015;526:68-74.
494 doi:10.1038/nature15393.
495 30. Zhu Z, Zhang F, Hu H, Bakshi A, Robinson MR, Powell JE, Montgomery GW,
496 Goddard ME, Wray NR, Visscher PM, et al. Integration of summary data from
497 GWAS and eQTL studies predicts complex trait gene targets. Nat Genet.
498 2016;48:481-487. doi:10.1038/ng.3538.
499 31. Hemani G, Zheng J, Elsworth B, Wade KH, Haberland V, Baird D, Laurin C, Burgess
500 S, Bowden J, Langdon R, et al. The MR-Base platform supports systematic causal
501 inference across the human phenome. Elife. 2018;7:e34408. doi:10.7554/elife.34408.
502 32. Bowden J, Davey Smith G, Burgess S. Mendelian randomization with invalid
503 instruments: effect estimation and bias detection through Egger regression. Int J
504 Epidemiol. 2015;44:512-525. doi:10.1093/ije/dyv080.
505 33. Bowden J, Davey Smith G, Haycock PC, Burgess S. Consistent Estimation in
506 Mendelian Randomization with Some Invalid Instruments Using a Weighted Median
507 Estimator. Genet Epidemiol. 2016;40:304-314. doi:10.1002/gepi.21965.
508 34. Wallace C. Eliciting priors and relaxing the single causal variant assumption in
509 colocalisation analyses. PLoS Genet. 2020;16:e1008720.
510 doi:10.1371/journal.pgen.1008720.
511 35. GTEx Consortium. The GTEx Consortium atlas of genetic regulatory effects across
512 human tissues. Science. 2020;369:1318-1330. doi:10.1126/science.aaz1776.

36. Colobran R, Armengol Mdel P, Faner R, Gärtner M, Tykocinski LO, Lucas A, Ruiz M, Juan M, Kyewski B, Pujol-Borrell R. Association of an SNP with intrathymic transcription of TSHR and Graves' disease: a role for defective thymic tolerance. Hum Mol Genet. 2011;20:3415-3423. doi:10.1093/hmg/ddr247.
37. Marín-Sánchez A, Álvarez-Sierra D, González O, Lucas-Martin A, Sellés-Sánchez A, Rudilla F, Enrich E, Colobran R, Pujol-Borrell R. Regulation of TSHR Expression in the Thyroid and Thymus May Contribute to TSHR Tolerance Failure in Graves' Disease Patients via Two Distinct Mechanisms. Front Immunol. 2019;10:1695. doi:10.3389/fimmu.2019.01695.
38. Chen CR, Pichurin P, Nagayama Y, Latrofa F, Rapoport B, McLachlan SM. The thyrotropin receptor autoantigen in Graves disease is the culprit as well as the victim. J Clin Invest. 2003;111:1897-1904. doi:10.1172/JCI17069.
39. Berchner-Pfannschmidt U, Moshkelgosha S, Diaz-Cano S, Edelmann B, Görtz GE, Horstmann M, Noble A, Hansen W, Eckstein A, Banga JP. Comparative Assessment of Female Mouse Model of Graves' Orbitopathy Under Different Environments, Accompanied by Proinflammatory Cytokine and T-Cell Responses to Thyrotropin Hormone Receptor Antigen. Endocrinology. 2016;157:1673-1682. doi:10.1210/en.2015-1829.
40. Daser A, Gulbins A, Horstmann M, Kalisch J, Nolte I, Hashemi Arani F, Lang S, Mattheis S, Bechrakis NE, Banga JP, et al. TNFα drives disease-specific chemokine networks and orbital tissue remodeling in a preclinical model of thyroid eye disease. Eur Thyroid J. 2026;15:ETJ250407. doi:10.1530/ETJ-25-0407.
41. Eckstein AK, Lösch C, Glowacka D, Schott M, Mann K, Esser J, Morgenthaler NG. Euthyroid and primarily hypothyroid patients develop milder and significantly more

asymmetrical Graves ophthalmopathy. Br J Ophthalmol. 2009;93:1052-1056. doi:10.1136/bjo.2007.137265.

42. Douglas RS, Couch S, Wester ST, Fowler BT, Liu CY, Subramanian PS, Tang R, Nguyen QT, Maamari RN, Ugradar S, et al. Efficacy and Safety of Teprotumumab in Patients With Thyroid Eye Disease of Long Duration and Low Disease Activity. J Clin Endocrinol Metab. 2024;109:25-35. doi:10.1210/clinem/dgad637.

43. Wallace C. A more accurate method for colocalisation analysis allowing for multiple causal variants. PLoS Genet. 2021;17:e1009440. doi:10.1371/journal.pgen.1009440.

Figure legends

Figure 1. Study design and gene-level discovery in BBJ. (A) Analysis flow. Blood expression instruments from eQTLGen were tested against BBJ GD for discovery; UKB hyperthyroidism and FinnGen GO were the original comparison datasets, and FinnGen GD, FinnGen GO strict and the GTEx tissue extension were added post hoc. The three FinnGen endpoints overlap and are not independent replications. (B) Each grey point is one estimable gene (n = 2,234), placed at the GRCh37 position of its strongest selected expression instrument and plotted by its BBJ MR *P* value; points are genes, not single variants. Filled black points are the 13 genes that passed $P < 0.05/2{,}544$ (dashed line, $-\log_{10}P \approx 4.71$); the open circle is the focal gene *IGF1R*, which did not. The threshold applies to the BBJ discovery screen, not to later exploratory analyses. GTEx, Genotype-Tissue Expression; MR, Mendelian randomization; SNP, single-nucleotide polymorphism; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Figure 2. *TSHR*, *IGF1R* and *CTLA4* across five disease datasets. (A) MR odds ratios with 95% CIs per approximately 1 SD higher genetically predicted blood expression: Wald ratio for one instrument, multiplicative random-effects IVW for several. The axis is logarithmic and the dashed line is OR = 1. *TSHR* used one instrument, rs179252, in every dataset. Colours identify datasets. (B) Colocalization posterior probabilities for the same comparisons ($p1 = p2 = 10^{-4}$, $p12 = 10^{-5}$), stacked from zero as H4 (one shared variant), H3 (distinct variants), H2 (expression signal only) and H0/H1 (neither trait, or disease only). The dotted line marks PP.H4 = 0.80, the descriptive reporting threshold. Estimates, *P* values and posterior probabilities are given in Table 2; *P* values and posterior probabilities answer different questions. FinnGen GO controls include people with GD but no recorded eye disease. CI, confidence interval; IVW, inverse-variance weighted; MR, Mendelian

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Figure 3. Colocalization across 24 selected genes and *TSHR* prior sensitivity. (A) PP.H4 for each gene and dataset (p1 = p2 = 10^{-4}, p12 = 10^{-5}); shading follows the colour bar. Boxed cells have PP.H4 ≥ 0.80, and a dot marks cells that remain ≥ 0.80 at p12 = 10^{-6}. Horizontal lines separate *TSHR*, *IGF1R* and *CTLA4*; the other 11 BBJ discoveries; the nine additional FinnGen GD discoveries; and *HLA-G*, added because its MR result changed with the allele-frequency source. Only these 24 genes underwent colocalization. Values below 0.001 are shown as <0.001; full-precision values are in the Supplementary Dataset. Low BBJ PP.H4 for the two *IGLV* genes partly reflects absence of the lead expression variant from the BBJ data. (B) *TSHR* PP.H4 in each dataset at three shared-variant priors (p12); points are offset slightly to avoid overlap, and the dashed line marks 0.80. PP.H4 is a posterior probability under the colocalization model, not a *P* value. MR, Mendelian randomization; PP.H4, posterior probability of one shared causal variant; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table 1. Data sources and their roles

Dataset / endpoint	Cases	Controls	Role
eQTLGen blood cis-eQTL	—	—	Exposure; up to 31,684 participants (median 28,092 per variant); predominantly European
BBJ GD (GCST90018627)	2,809	172,656	Discovery; East Asian
FinnGen GD (E4_GRAVES_STRICT)	3,962	496,386	Post hoc same-disease comparison; Finnish
UKB hyperthyroidism (GCST90038636)	3,731	480,867	Broader-phenotype comparison; self-reported hyperthyroidism or thyrotoxicosis; ancestry not established as exclusively European
FinnGen GO (GRAVES_OPHT)	858	499,490	Original ophthalmopathy (TED-enriched) comparison; Finnish
FinnGen GO strict (E4_GRAVES_OPHT_STRICT)	753	499,595	Post hoc ophthalmopathy definition sensitivity; Finnish
GTEx v8 tissue eQTL	—	—	Post hoc tissue extension (thyroid, cultured fibroblasts, subcutaneous and visceral adipose, whole blood; n = 469–670; Table S4)
1000 Genomes Phase 3 European reference (n = 503)	—	—	LD clumping of instruments and reference allele frequencies

The three FinnGen R12 endpoints share participants and are not independent replications. FinnGen ophthalmopathy controls are all other participants, including people with GD but no recorded eye disease, so GO estimates do not estimate TED risk among patients with GD. UKB hyperthyroidism is broader than GD. ‘Strict’ is the FinnGen endpoint label, not proof of perfect specificity. The descriptive orbital RNA-sequencing series (four TED patients, one control) is separate from these datasets. eQTL, expression quantitative trait locus; GTEx, Genotype-Tissue Expression; LD, linkage disequilibrium; TED, thyroid eye disease; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table 2. MR and colocalization results for *TSHR*, *IGF1R* and *CTLA4* across five disease datasets

Gene	Dataset	IVs	OR (95% CI)	*P* value	Q-test *P*	PP.H2	PP.H3	PP.H4	PP.H4, p12 = 10^{-6}
TSHR	BBJ GD	1	0.12 (0.07–0.21)	1.09×10^{-14}*†	NA	<0.001	0.049	**0.951**	0.661
TSHR	FinnGen GD	1	0.08 (0.05–0.12)	3.19×10^{-32}*†	NA	<0.001	0.736	0.264	0.035
TSHR	UKB hyperthyroidism	1	0.09 (0.06–0.14)	8.77×10^{-28}*†	NA	<0.001	0.774	0.226	0.028
TSHR	FinnGen GO	1	0.10 (0.04–0.24)	2.82×10^{-7}*†	NA	<0.001	0.014	**0.986**	**0.875**
TSHR	FinnGen GO strict	1	0.15 (0.06–0.39)	9.02×10^{-5}*	NA	0.027	0.082	**0.891**	0.451
IGF1R	BBJ GD	4	1.56 (1.07–2.28)	0.0212*	0.902	0.690	0.236	0.073	0.008
IGF1R	FinnGen GD	3	1.07 (0.86–1.33)	0.539	0.882	0.731	0.259	0.010	0.001
IGF1R	UKB hyperthyroidism	4	1.35 (1.07–1.70)	0.0117*	0.250	0.400	0.196	0.404	0.063
IGF1R	FinnGen GO	3	1.41 (0.85–2.33)	0.182	0.309	0.623	0.346	0.032	0.003
IGF1R	FinnGen GO strict	3	1.52 (0.92–2.49)	0.0996	0.409	0.571	0.391	0.038	0.004
CTLA4	BBJ GD	1	0.18 (0.11–0.27)	5.45×10^{-15}*†	NA	<0.001	0.799	0.201	0.025
CTLA4	FinnGen GD	2	0.17 (0.08–0.36)	2.15×10^{-6}*†	0.0275	<0.001	0.746	0.254	0.033
CTLA4	UKB hyperthyroidism	2	0.21 (0.08–0.57)	0.00220*	0.00131	<0.001	0.047	**0.953**	0.672
CTLA4	FinnGen GO	2	0.17 (0.04–0.68)	0.0118*	0.0486	<0.001	0.022	**0.978**	**0.816**
CTLA4	FinnGen GO strict	2	0.17 (0.05–0.64)	0.00888*	0.0755	<0.001	0.021	**0.979**	**0.820**

MR estimates are ORs per approximately one SD higher genetically predicted blood expression: Wald ratio for one IV, multiplicative random-effects IVW for several. *TSHR* used one instrument, rs179252, in every dataset; its G allele raises blood expression by 0.105 SD per allele (F = 176), and its per-allele disease ORs were 0.77–0.82 (Table S3). The per-SD ORs therefore rescale a moderate per-allele association, and each single-IV *P* value equals the variant's disease-association z-test *P* value. *CTLA4* multi-IV estimates are concentrated in rs13030124, and three of the four have Q-test $P < 0.05$; in BBJ and UKB, two of four single-instrument omissions removed nominal *IGF1R* significance (Figure S2). * Nominal $P < 0.05$; † $P < 0.05/2{,}544$, the screen threshold, applied descriptively outside BBJ. PP.H2–H4 are colocalization posterior probabilities (H2, expression signal only; H3, distinct variants; H4, one shared variant) with $p1 = p2 = 10^{-4}$ and $p12 = 10^{-5}$ unless stated; bold, ≥0.80. They are not *P* values. BBJ colocalization pairs European eQTL with East Asian disease statistics. FinnGen ophthalmopathy controls are all other participants, including people with GD but no recorded eye disease, so GO estimates do not estimate TED risk among patients with GD. CI, confidence interval; eQTL, expression quantitative trait locus; IV, instrumental variable; IVW, inverse-variance weighted; MR, Mendelian randomization; NA, not applicable (one IV); OR, odds ratio; PP, posterior probability; SD, standard deviation; TED, thyroid eye disease; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table 3. Thirteen BBJ discoveries and their FinnGen GD comparison

BBJ discovery gene	Chr	IVs, BBJ/FinnGen GD	BBJ OR (95% CI)	BBJ *P*	FinnGen GD OR (95% CI)	FinnGen GD *P*	Focused support	FinnGen GD PP.H4
HLA-A	6 (MHC)	4/4	1.98 (1.73–2.27)	2.60×10^{-23}	1.08 (0.77–1.53)	0.648	No	<0.001
HLA-DQA2	6 (MHC)	1/1	2.39 (1.95–2.93)	8.30×10^{-17}	1.90 (1.64–2.20)	1.41×10^{-17}	Yes	<0.001
CTLA4	2	1/2	0.18 (0.11–0.27)	5.45×10^{-15}	0.17 (0.08–0.36)	2.15×10^{-6}	Yes	0.254
TSHR	14	1/1	0.12 (0.07–0.21)	1.09×10^{-14}	0.08 (0.05–0.12)	3.19×10^{-32}	Yes	0.264
C4A	6 (MHC)	1/3	0.45 (0.35–0.57)	2.23×10^{-10}	0.41 (0.26–0.63)	4.97×10^{-5}	Yes	<0.001
HSD3B7	16 (16p11.2 cluster)	1/2	0.29 (0.18–0.47)	2.04×10^{-7}	0.90 (0.72–1.13)	0.372	No	0.011
TUBB	6 (MHC)	2/3	1.49 (1.28–1.74)	3.14×10^{-7}	3.28 (2.09–5.17)	2.68×10^{-7}	Yes	<0.001
VKORC1	16 (16p11.2 cluster)	1/0	0.13 (0.06–0.29)	6.35×10^{-7}	NA	NA	NA	0.011
TNFSF14	19	1/1	0.63 (0.52–0.76)	1.48×10^{-6}	0.77 (0.65–0.92)	0.00361	Yes	0.303
PRSS36	16 (16p11.2 cluster)	1/1	6.00 (2.87–12.57)	2.02×10^{-6}	1.40 (1.00–1.96)	0.0490	No	0.063
MAPKAPK5	12	4/4	0.977 (0.967–0.987)	5.28×10^{-6}	0.990 (0.929–1.055)	0.751	No	0.041
PSMB8	6 (MHC)	1/1	1.25 (1.13–1.38)	6.86×10^{-6}	0.96 (0.85–1.08)	0.512	No	<0.001
IFNGR1	6	1/1	2.10 (1.51–2.91)	9.41×10^{-6}	1.62 (1.18–2.24)	0.00330	Yes	0.313

Genes passed the BBJ screen threshold ($P < 0.05/2{,}544$) and are listed by BBJ *P* value. Focused support in FinnGen GD requires the same direction and $P < 0.05/13$; 7 of 13 qualify. IVs are counted separately for each dataset because instrument availability differs. The five MHC genes and the three chromosome 16p11.2 genes are not independent signals; the latter are compatible with one regional association. *MAPKAPK5* is shown to three decimals to preserve its small effect. NA, not estimable (no retained instrument), not a negative result; *VKORC1* colocalization does not require an MR instrument. PP.H4, shared-variant posterior probability under $p12 = 10^{-5}$; bold, ≥0.80. Chr, chromosome of the strongest instrument; MHC, major histocompatibility complex; CI, confidence interval; IV, instrumental variable; MR, Mendelian randomization; OR, odds ratio; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Figure 1

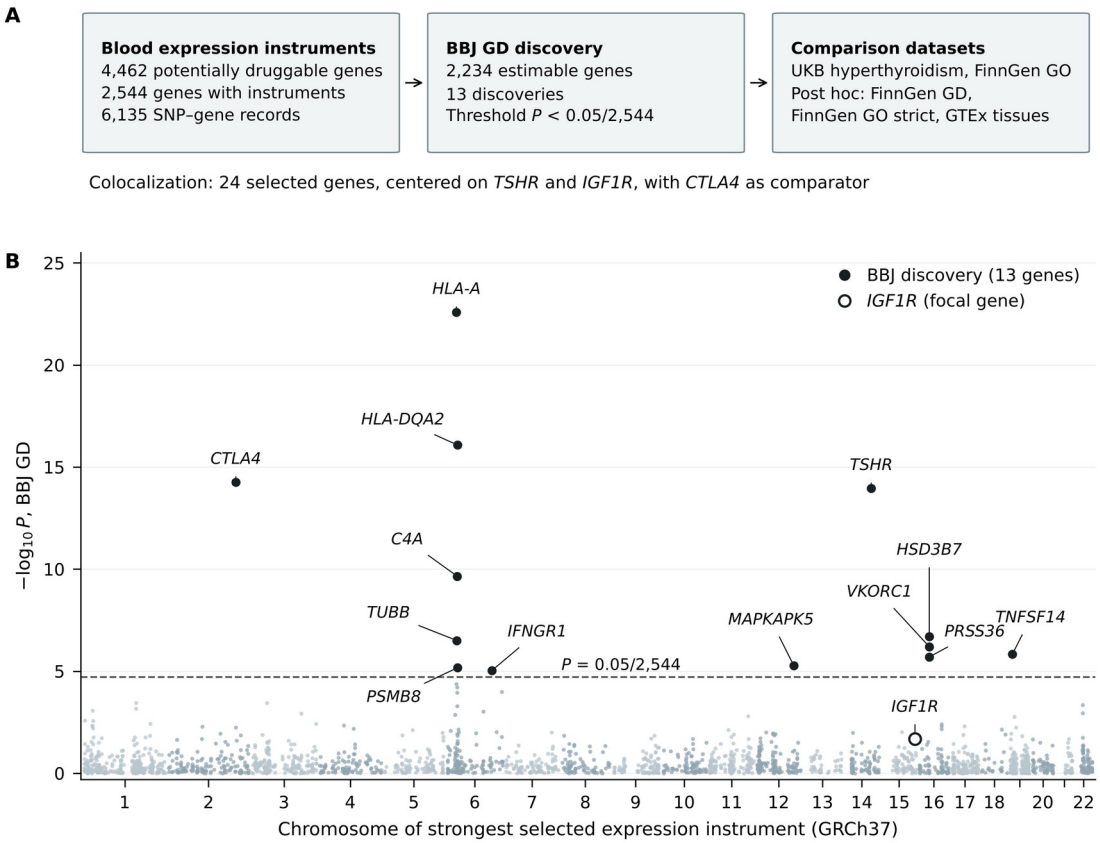


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Figure 2

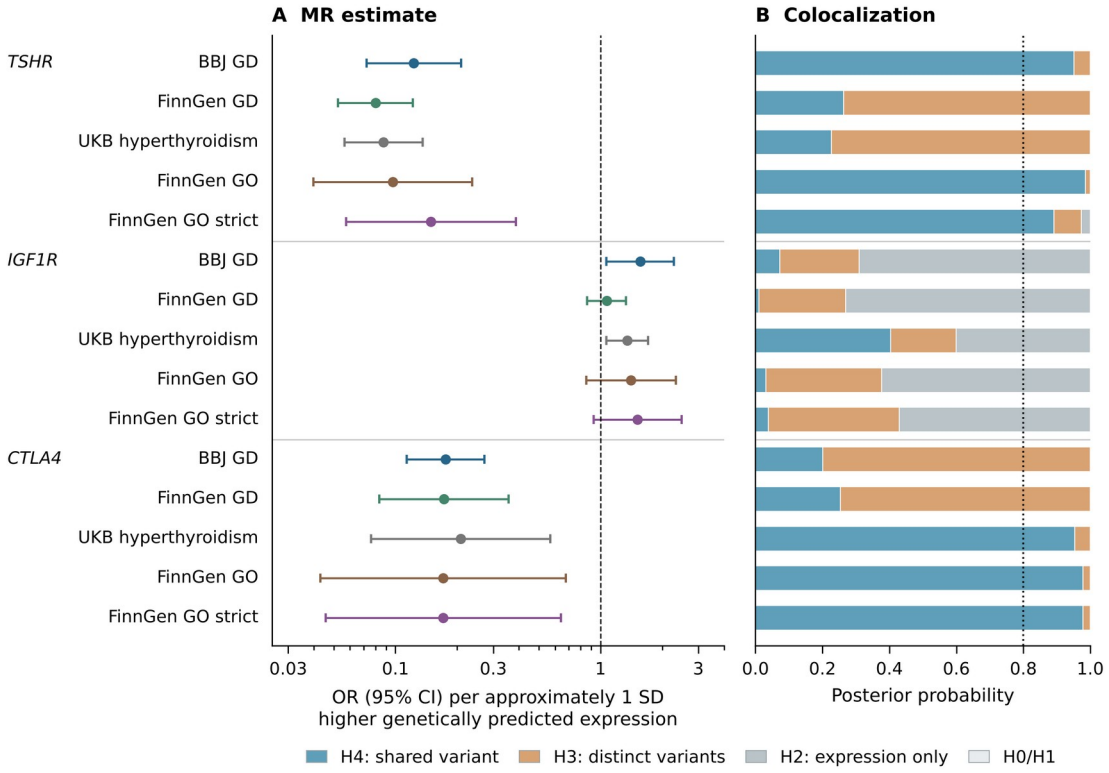


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Figure 3

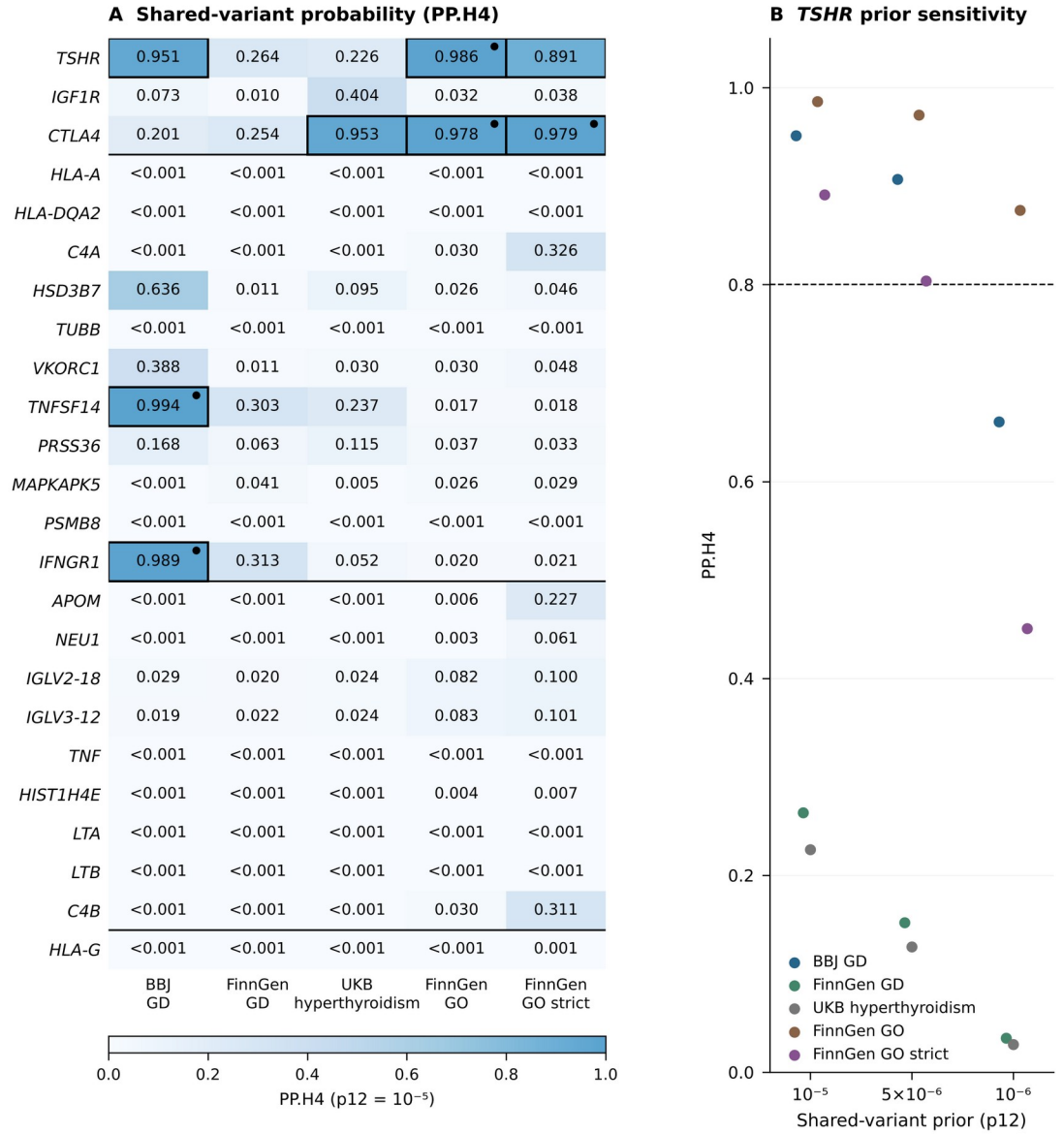


Figure 3. Colocalization across 24 selected genes and *TSHR* prior sensitivity. (A) PP.H4 for each gene and dataset (p1 = p2 = 10^{-4}, p12 = 10^{-5}); shading follows the colour bar. Boxed cells have PP.H4 ≥ 0.80, and a dot marks cells that remain ≥ 0.80 at p12 = 10^{-6}. Horizontal lines separate *TSHR*, *IGF1R* and *CTLA4*; the other 11 BBJ discoveries; the nine additional FinnGen GD discoveries; and *HLA-G*, added because its MR result changed with the allele-frequency source. Only these 24 genes underwent colocalization. Values below 0.001 are shown as <0.001; full-precision values are in the Supplementary Dataset. Low BBJ PP.H4 for the two *IGLV* genes partly reflects absence of the lead expression variant from the BBJ data. (B) *TSHR* PP.H4 in each dataset at three shared-variant priors (p12); points are offset slightly to avoid overlap, and the dashed line marks 0.80. PP.H4 is a posterior probability under the colocalization model, not a *P* value. MR, Mendelian randomization; PP.H4, posterior probability of one shared causal variant; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Supplementary material

TSHR and IGF1R in Graves disease and thyroid eye disease: genetic evidence from blood expression

Supplementary methods

Dataset definitions and analysis chronology

BBJ GD (GCST90018627) and UKB self-reported hyperthyroidism/thyrotoxicosis (GCST90038636) retained the source sample sizes and estimates. UKB used BOLT-LMM linear-model statistics; its report included adjustment for sex, age, BMI, assessment centre, ethnicity, genotyping batch and 20 principal components. The available documentation did not establish an exclusively European analysis sample. FinnGen R12 GWAS counts were 858/499,490 for GRAVES_OPHT, 3,962/496,386 for E4_GRAVES_STRICT and 753/499,595 for E4_GRAVES_OPHT_STRICT. Counts refer to the analyzed GWAS files, not later phenotype-browser totals. Strict GD combines specified GD/ophthalmopathy diagnostic criteria with the Graves condition and exclusion of amiodarone exposure; strict ophthalmopathy requires its specified ophthalmopathy and thyrotoxicosis codes. BBJ cases merged physician diagnoses with curated medical histories and text-mined records; controls lacked GD or related diagnoses; covariates were age, age^2, sex, their interactions and 20 principal components. None supplies GD-only controls.

The original three-outcome analysis preceded the two FinnGen extensions. The expanded analysis plan was recorded before the new endpoint associations were computed, but it was not a prospective registration of the complete study. The 24 colocalization genes comprise 13 BBJ discoveries, *IGF1R*, nine additional FinnGen GD screen discoveries, and frequency-sensitive *HLA-G*. No new discovery threshold was optimized from these results.

Instrument selection and effect reconstruction

Of 4,479 druggable-genome genes, 4,462 mapped to GRCh37. Significant cis-eQTL instruments ($P < 5\times10^{-8}$ within ±1 Mb of the gene) were clumped at $r^2 < 0.001$ within 10 Mb using the European 1000 Genomes Phase 3 reference (n = 503). $SE = 1/\sqrt{[2f(1-f)(N + Z^2)]}$ and $\beta = Z \times SE$, where f is the assessed-allele frequency and N is the variant-specific expression sample size (median 28,092; range 232–31,684). Unit expression variance was assumed. Variant $F = Z^2$. The estimate is approximately per expression SD, not per SD of the genetic predictor.

TwoSampleMR harmonise_data(action = 2) aligned alleles and excluded ambiguous palindromic variants with MAF > 0.42. Missing outcome estimates were not imputed. BBJ and FinnGen retained log-odds scales; UKB β and SE were each divided once by $\mu(1-\mu)$, with $\mu = 3,731/484,598$. GRCh37 expression and GRCh38 FinnGen coordinates were not directly joined: identifiers and allele pairs

were checked, with duplicate and alias audits. Source covariate adjustments were retained; patient-level re-adjustment and subgroup analysis were unavailable.

MR estimators and instrument diagnostics

For a single instrument, the Wald coefficient is β_Y/β_X and its first-order SE is $SE_Y/|\beta_X|$. Multiple-instrument MR used inverse-variance weighted regression through the origin. Multiplicative random-effects SEs included under-dispersion correction, so heterogeneity could widen the interval. Normal two-sided tests and 95% CIs were used for the primary estimators. The alternative-estimator worksheet preserves weighted median, weighted mode and MR-Egger results for the original three outcomes; mode and Egger intervals use the corresponding t distributions. These alternative estimators were not rerun for all new endpoints. With three or four *IGF1R* instruments, these estimators had little power. No Steiger directionality test was performed.

Post hoc leave-one-out analyses applied IVW when at least two variants remained and the Wald ratio when one remained. Single-instrument *TSHR* and BBJ *CTLA4* were ineligible. Original three-outcome results, new-endpoint results for the fixed 14-gene set, and 72 *C4B*/*HLA-G* omission estimates under two frequency sources are retained in separately named worksheets. Independent calculations reproduced the eight full-set and 72 omission estimates for *C4B* and *HLA-G*. They reuse the same association data and are not independent replications. *C4B*/*HLA-G* instrument weights use β_X^2/SE_Y^2, normalized within each full model. A single-SNP *P* value can be much smaller than the full IVW *P* value when heterogeneity inflates the full-model SE.

TSHR clumping sensitivity used eQTL thresholds of $5\times10^{-8}/5\times10^{-6}$, r^2 thresholds of 0.001/0.01/0.1, and European (n = 503) or East Asian (n = 504) reference panels. Primary settings selected one European and two East Asian variants, and rs179252 was retained in every setting. Chromosome 16p11.2 analyses found high East Asian LD among rs4889606, rs34649473 and rs78924645; GCTA 1.95.1 COJO stepwise selection (BBJ; East Asian reference) retained one signal, rs8050588; conditional on it, the three variants had *P* = 0.81–0.93, supporting regional dependence rather than three independent gene effects.

TSHR allele alignment and reference LD

In a post hoc audit, original full eQTLGen and disease GWAS files were re-extracted for rs179252 and rs179247. Their GRCh37 positions were 14:81435985 and 14:81432546, and GRCh38 positions were 14:80969641 and 14:80966202. Both non-palindromic allele pairs matched across sources. UKB effects were reversed to the G allele after the existing scale conversion; BBJ and FinnGen required no reversal. Exposure coefficients and first-order Wald estimates were independently reconstructed. Original GWAS *P* values and recalculated MR *P* values were retained separately.

G-allele counts at both variants were positively correlated in European (n = 503; r = 0.978322812; r^2 = 0.957115524) and East Asian (n = 504; r = 0.995803919; r^2 = 0.991625445) references, without missing genotypes. Direct BED decoding agreed with PLINK dosage output. PLINK haplotype estimates supported G–G and T–A coupling; these were inferred from unphased genotypes, not directly observed phased haplotypes. Alignment to the G allele resolved the negative default European correlation obtained when counting T at rs179252. The TSHR_allele_audit worksheet retains both eQTL records, both variants across five disease outcomes, all four inferred haplotypes in each population, and MR reproduction. This audit does not identify a causal variant or establish mediation in thymus or orbit.

Colocalization hypotheses and regional inputs

coloc.abf received beta/varbeta data for disease (dataset 1, case-control type) and expression (dataset 2, quantitative type, sdY = 1), with the default prior effect SDs of 0.2 and 0.15, respectively. Outcome allele frequency was not used in the Bayes factors. Complete available cis-eQTL statistics, not just significant eQTLs, were matched by identifier and allele compatibility. H0 denotes neither trait associated; H1 disease only; H2 expression only; H3 both traits associated through distinct variants; H4 both traits associated through one shared variant. PP.Hk is the posterior probability of hypothesis k, conditional on the model and priors.

Primary priors were p1 = p2 = 10^{-4} and p12 = 10^{-5}; sensitivity p12 values were 5×10^{-6} and 10^{-6}. The PP.H4 ≥ 0.80 threshold was a reporting convention, not a significance test. H3 is a competing distinct-variant model, not a count of causal signals. Multi-signal colocalization was not performed.

Four frequency/availability scenarios were used: original_reference (original available reference frequencies), paired_reference and paired_eqtlgen (the same variants with the respective frequencies), and available_eqtlgen (all matching variants with valid cohort frequencies). There are 1,440 gene–outcome–scenario–prior rows; no PP.H4 changed side of 0.80 across scenarios. The additional *TSHR* analysis comprises four windows × five outcomes × four scenarios × three priors = 240 rows. Windows were the full source cis-region, ±500 kb and ±250 kb around the recorded GRCh37 GenePos anchor, and the five-outcome variant intersection. The full cis-source contained 7,309 eQTL rows; all three FinnGen full-region analyses matched 6,579 variants, and the five-outcome intersection contained 4,030. Raw outcome regional scans checked whether the strongest disease variants survived matching. New candidate coverage checks also examined marginal expression strength and shared instruments; the absent rs5759410 lead in BBJ was checked against variant aliases without finding a replacement.

For *TSHR*, *IGF1R* and *CTLA4*, colocalization and MR in FinnGen GD and strict ophthalmopathy were independently reproduced from re-downloaded FinnGen R12 regional statistics using two

separately written implementations, with the original ophthalmopathy endpoint as a control. The allele-compatible variant sets were the same (6,675, 7,850 and 4,993 variants), and the top variant was the same in all 27 gene–endpoint–prior settings, including the control. The reproduction used gnomAD v2.1.1 European frequencies, which were available for more of these variants than the 1000 Genomes European frequencies used in the reported analysis (6,674, 7,850 and 4,992 analysed, versus 6,579, 7,740 and 4,924). On the control endpoint, PP.H4 changed by at most 4.1×10^{-4}; for *TSHR* this difference came from the additional variants. Reported new-endpoint PP.H4 values were reproduced within 1.2×10^{-3}, and no setting changed side of 0.80. The *TSHR* strict-ophthalmopathy value at p12 = 10^{-6} was 0.449 rather than 0.451 (0.451 on the reported variant set), and *IGF1R* PP.H2 and PP.H3 differed by up to 0.003, as on the control endpoint. MR estimates were reproduced to numerical precision on the original exposure scale.

Frequency substitution and precision benchmarks

The published eQTLGen frequency release covered 26,609 participants, excluding Framingham. Reconstructing β_X and SE_X with fixed Z and N preserves their ratio up to floating-point precision. For a single-IV Wald estimate, frequency scaling changes the effect unit and SE together and preserves the normal-test *P* value; for IVW, variant weights can change. The new-endpoint MR dataset separates identical-variant scaling from repeat allele harmonization. *C4B* and *HLA-G* were audited with identical retained variants and outcome effects, so their differences arise from scaling and weighting rather than a changed variant set. Large frequency discrepancies are documented, not assumed to be resolved by correct identifier matching.

The detection benchmarks use $(z_{1-\alpha/2} + z_{0.80}) \times SE$ as the approximate 80% minimum detectable absolute log-OR. Alpha was 0.05/2,544 for BBJ and 0.05 for the original UKB/FinnGen comparisons. These are precision summaries using observed SEs, not prospective sample-size calculations, post hoc proof of power, or colocalization power. The Detection_original worksheet retains these three-outcome results. Applying the same formula to the MR_primary standard errors for FinnGen GD at alpha 0.05/2,544 gave 24.8% and 52.2% of 2,480 estimable genes with 80% power for ORs of 1.5 and 2.0. At alpha 0.05, the detectable *IGF1R* ORs were 1.72 in BBJ, 1.39 in UKB and 1.36 in FinnGen GD.

Fixed-panel tissue extension

The post hoc tissue extension retained a frozen panel of *TSHR* and *IGF1R* in five standard-release GTEx v8 tissues. QTL sample counts were 574 (thyroid), 483 (cultured fibroblasts), 581 (subcutaneous adipose), 469 (visceral adipose) and 670 (whole blood). The original v8 archive supplied significant variant–gene pairs and eGenes records for all tested genes; gene-specific release thresholds exceeded 5×10^{-8} for every eligible combination, permitting complete selection at the fixed

threshold. Absence from the tested-gene file was distinguished from a minimum eQTL *P* value above the threshold. The standard release includes multiple ancestries; the overall donor cohort was 85.3% European American, not a tissue-specific percentage.

Variants were restricted to ±1 Mb of the source GENCODE v26 TSS, $P < 5\times10^{-8}$, F > 10 and MAF ≥ 0.01. GTEx variant IDs identify GRCh38 REF and ALT, with slope referring to ALT. Actual effect-allele frequencies were reconstructed from each selected eGene MAF and ref_factor, not by treating MAF as ALT frequency. GTEx mappings linked GRCh38 variants to rsIDs and GRCh37 reference coordinates. PLINK clumping used 503 European reference samples, $r^2 < 0.001$ and a 10-Mb window; an independent genotype-based calculation reproduced selection. One of 20 thyroid *IGF1R* candidates was absent from the reference, while its strongest candidate was retained. All selected variants were non-palindromic and matched exactly in the five disease datasets; no proxies or relaxed thresholds were used.

Each of the four estimable gene–tissue combinations retained one SNP. Wald estimates used β_Y/β_X and $SE_Y/|\beta_X|$ with normal two-sided *P* values and 95% CIs; independent calculations reproduced all 20 estimates. The UKB linear-to-log-odds conversion was applied once. ORs represent one unit of inverse-normal-transformed normalized expression; they are not directly comparable to an intervention or to the reconstructed eQTLGen scale. Primary adjusted $P = \min(1, 10P)$ for FinnGen GD; secondary adjusted $P = \min(1, 40P)$ for the other datasets. The 30 unestimable cells remained in the planned families. Single-instrument analyses cannot support heterogeneity, leave-one-out or MR-Egger diagnostics. Significant-only eQTL pairs were not used for colocalization; full regional data were not obtained from the requester-pays resource, and no tissue posterior was calculated. Tissue_extension in the single Supplementary Dataset retains all 50 cells, availability, selected and excluded candidates, harmonized alleles and source statistics.

Orbital expression and reproducibility

Orbital adipose tissue was collected during orbital decompression from four patients with inactive TED; the control was nasal fat pad from one person without thyroid disease undergoing cosmetic upper blepharoplasty. Paired-end RNA sequencing (Illumina NovaSeq) used two technical-replicate libraries per donor, which were combined by biological sample; reads were trimmed (Trimmomatic 0.38), aligned to GRCh38 (HISAT2 2.1.0) and quantified as TPM (StringTie 2.1.3b). Figure S1 shows sample-level transcript abundance and $\log_2[(\text{mean TED TPM} + 0.01)/(\text{control TPM} + 0.01)]$. One control cannot estimate control-group biological variance reliably, and no differential-expression *P* value was used. The dataset is descriptive and did not influence candidate selection.

The workbook Readme and Column_dictionary sheets describe every sheet; reproduction validates implementation, not causal assumptions.

Table S1. Additional FinnGen GD screen discoveries

Gene	Chr	FinnGen GD IVs	FinnGen GD OR (95% CI)	FinnGen GD *P*	BBJ PP.H4	FinnGen GD PP.H4	FinnGen GO strict PP.H4
APOM	6 (MHC)	1	0.05 (0.04–0.06)	1.43×10^{-108}	<0.001	<0.001	0.227
NEU1	6 (MHC)	1	0.04 (0.03–0.06)	1.43×10^{-108}	<0.001	<0.001	0.061
IGLV2-18	22	1	0.56 (0.48–0.66)	7.52×10^{-12}	0.029	0.020	0.100
IGLV3-12	22	1	1.20 (1.14–1.26)	7.52×10^{-12}	0.019	0.022	0.101
TNF	6 (MHC)	1	0.60 (0.51–0.71)	4.65×10^{-10}	<0.001	<0.001	<0.001
HIST1H4E	6 (MHC)	1	2.54 (1.88–3.43)	1.51×10^{-9}	<0.001	<0.001	0.007
LTA	6 (MHC)	1	0.46 (0.35–0.61)	3.75×10^{-8}	<0.001	<0.001	<0.001
LTB	6 (MHC)	1	0.18 (0.10–0.36)	4.89×10^{-7}	<0.001	<0.001	<0.001
C4B	6 (MHC)	2	1.63 (1.31–2.03)	1.10×10^{-5}	<0.001	<0.001	0.311

Genes outside the BBJ discovery set that passed $P < 0.05/2{,}544$ in the post hoc full FinnGen GD screen (1000 Genomes European reference frequencies). These are not independent of the screen and seven of nine are in the MHC. *APOM*/*NEU1* share rs3101018 and *IGLV2-18*/*IGLV3-12* share rs5759410. The lead *IGLV* expression variant is absent in BBJ, weakening interpretation of its low PP.H4. None reached PP.H4 ≥0.80 ($p12 = 10^{-5}$) in any dataset. Chr, chromosome; CI, confidence interval; IV, instrumental variable; MHC, major histocompatibility complex; OR, odds ratio; PP.H4, shared-variant posterior probability; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table S2. Frequency-sensitive exploratory candidates

Gene	Dataset	Frequency source	IVs	OR (95% CI)	*P* value	Q-test *P*
C4B	FinnGen GD	1000G EUR	2	1.63 (1.31–2.03)	1.10×10^{-5}	7.79×10^{-7}
C4B	FinnGen GD	eQTLGen cohort	2	1.75 (1.31–2.33)	1.48×10^{-4}	1.36×10^{-8}
C4B	FinnGen GO strict	1000G EUR	2	1.68 (1.44–1.95)	2.74×10^{-11}	0.122
C4B	FinnGen GO strict	eQTLGen cohort	2	1.80 (1.46–2.23)	6.17×10^{-8}	0.0587
HLA-G	FinnGen GD	1000G EUR	16	0.86 (0.72–1.03)	0.107	9.98×10^{-24}
HLA-G	FinnGen GD	eQTLGen cohort	16	0.60 (0.47–0.75)	1.72×10^{-5}	2.26×10^{-10}
HLA-G	FinnGen GO strict	1000G EUR	16	0.91 (0.76–1.08)	0.267	0.0231
HLA-G	FinnGen GO strict	eQTLGen cohort	16	0.70 (0.53–0.92)	0.0122	0.131

Exposure effects were reconstructed with 1000 Genomes Phase 3 European reference allele frequencies (1000G EUR; primary) or eQTLGen cohort frequencies; variant sets and outcome effects were held fixed, so changes reflect rescaled expression effects and IVW weights. *HLA-G* was added because its eQTLGen-frequency *P* crossed the screen threshold ($P < 0.05/2{,}544$). Q tests heterogeneity, not directional pleiotropy; its *P* value is not a gene-association *P* value. All 72 omission estimates and instrument weights are in the workbook. CI, confidence interval; IV, instrumental variable; IVW, inverse-variance weighted; OR, odds ratio; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table S3. *TSHR* regional colocalization and prior sensitivity

Dataset	Matched variants	Disease lead (*P*)	rs179252 *P*	rs179252-G OR (95% CI)	H4 top variant	PP.H4, 10^{-5} / 10^{-6}	PP.H4 range across windows
BBJ GD	4,336	rs28414437 (1.83×10^{-15})	1.09×10^{-14}	0.80 (0.76–0.85)	rs179252	**0.951** / 0.661	0.951–0.951
FinnGen GD	6,579	rs1023586 (1.58×10^{-36})	3.19×10^{-32}	0.77 (0.73–0.80)	rs1023586	0.264 / 0.035	0.264–0.264
UKB hyperthyroidism	5,667	rs4903961 (1.17×10^{-43})	8.77×10^{-28}	0.77 (0.74–0.81)	rs1023586	0.226 / 0.028	0.226–0.226
FinnGen GO	6,579	rs3783947 (1.23×10^{-7})	2.82×10^{-7}	0.78 (0.71–0.86)	rs179252	**0.986** / **0.875**	0.986–0.987
FinnGen GO strict	6,579	rs2110695 (1.34×10^{-5})	9.02×10^{-5}	0.82 (0.74–0.90)	rs179252	**0.891** / 0.451	0.891–0.905

Disease lead, the strongest disease-associated variant among the matched cis variants, with its *P* value; rs179252 *P*, z-test *P* of the aligned G-allele disease association (the UKB source BOLT-LMM *P* was 1.3×10^{-27}). H4 top variant, the variant with the highest posterior under the shared-variant hypothesis; it is not necessarily the disease lead. PP.H4 under p12 = 10^{-5} and 10^{-6} (p1 = p2 = 10^{-4}); bold, ≥0.80. Window range covers the full source cis-region, ±500 kb, ±250 kb and the five-outcome intersection at p12 = 10^{-5}; it is not a confidence interval. All 240 window settings are in TSHR_windows. BBJ pairs European eQTL with East Asian disease statistics. CI, confidence interval; eQTL, expression quantitative trait locus; OR, odds ratio; PP.H4, shared-variant posterior probability; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Table S4. *TSHR* and *IGF1R* tissue expression extension

A. Availability

Gene / tissue	GTEx N	Instrument (F)	Status
TSHR / Thyroid	574	—	No instrument (minimum eQTL $P = 9.60\times10^{-8}$)
IGF1R / Thyroid	574	rs1815009 (F = 61.1)	Estimable
TSHR / Cultured fibroblasts	483	—	Not tested by GTEx in this tissue
IGF1R / Cultured fibroblasts	483	rs6598538 (F = 33.8)	Estimable
TSHR / Subcutaneous adipose	581	rs12101255 (F = 38.5)	Estimable
IGF1R / Subcutaneous adipose	581	—	No instrument (minimum eQTL $P = 3.56\times10^{-7}$)
TSHR / Visceral adipose	469	rs74064796 (F = 32.1)	Estimable
IGF1R / Visceral adipose	469	—	No instrument (minimum eQTL $P = 7.76\times10^{-6}$)
TSHR / Whole blood	670	—	No instrument (minimum eQTL $P = 2.46\times10^{-4}$)
IGF1R / Whole blood	670	—	No instrument (minimum eQTL $P = 5.42\times10^{-5}$)

B. Estimates

Gene / tissue	Dataset	Family	OR (95% CI)	*P* value	Below family threshold	Direction vs blood
TSHR / Subcutaneous adipose	BBJ GD	Secondary	0.46 (0.38–0.55)	1.92×10^{-15}	Yes	Same
TSHR / Subcutaneous adipose	FinnGen GD	Primary	0.36 (0.31–0.43)	1.28×10^{-34}	Yes	Same
TSHR / Subcutaneous adipose	UKB hyperthyroidism	Secondary	0.31 (0.27–0.37)	2.86×10^{-42}	Yes	Same
TSHR / Subcutaneous adipose	FinnGen GO	Secondary	0.45 (0.32–0.63)	5.76×10^{-6}	Yes	Same
TSHR / Subcutaneous adipose	FinnGen GO strict	Secondary	0.51 (0.35–0.74)	3.67×10^{-4}	Yes	Same
TSHR / Visceral adipose	BBJ GD	Secondary	0.56 (0.46–0.67)	2.64×10^{-10}	Yes	Same
TSHR / Visceral adipose	FinnGen GD	Primary	0.50 (0.43–0.58)	4.09×10^{-19}	Yes	Same
TSHR / Visceral adipose	UKB hyperthyroidism	Secondary	0.39 (0.34–0.45)	9.19×10^{-38}	Yes	Same
TSHR / Visceral adipose	FinnGen GO	Secondary	0.69 (0.49–0.98)	0.0368	No	Same
TSHR / Visceral adipose	FinnGen GO strict	Secondary	0.78 (0.54–1.13)	0.186	No	Same
IGF1R / Thyroid	BBJ GD	Secondary	1.23 (0.90–1.68)	0.200	No	Same
IGF1R / Thyroid	FinnGen GD	Primary	0.76 (0.56–1.02)	0.0692	No	Opposite
IGF1R / Thyroid	UKB hyperthyroidism	Secondary	1.33 (0.99–1.80)	0.0594	No	Same
IGF1R / Thyroid	FinnGen GO	Secondary	1.02 (0.54–1.94)	0.945	No	Same
IGF1R / Thyroid	FinnGen GO strict	Secondary	1.23 (0.62–2.44)	0.549	No	Same
IGF1R / Cultured fibroblasts	BBJ GD	Secondary	0.91 (0.70–1.20)	0.511	No	Opposite
IGF1R / Cultured fibroblasts	FinnGen GD	Primary	1.11 (0.89–1.38)	0.359	No	Same
IGF1R / Cultured fibroblasts	UKB hyperthyroidism	Secondary	1.03 (0.82–1.29)	0.815	No	Same
IGF1R / Cultured fibroblasts	FinnGen GO	Secondary	0.78 (0.49–1.25)	0.301	No	Opposite
IGF1R / Cultured fibroblasts	FinnGen GO strict	Secondary	0.78 (0.47–1.30)	0.341	No	Opposite

Post hoc fixed-panel extension using GTEx v8; tissue colocalization was not performed. A, availability of the ten gene–tissue combinations; F statistics are for the single retained instrument. ‘Not tested’ means the gene is not among the genes GTEx tested in that tissue. GTEx whole blood (n = 670) yielded no instrument at the fixed threshold, whereas the main analysis used eQTLGen (up to 31,684 participants). B, all 20 estimates. The primary family is the ten FinnGen GD comparisons ($P < 0.005$); the secondary family is the 40 comparisons with the other datasets ($P < 0.00125$); unestimable combinations stay in both denominators. ORs are per unit of inverse-normal-transformed tissue expression and are not comparable in size with the blood ORs. With one instrument, each *P* value equals that variant’s disease-association z-test *P* value (UKB source BOLT-LMM *P* values differ slightly). The adipose *TSHR* instruments (rs12101255, rs74064796; 50 bp apart in *TSHR* intron 1) are themselves blood *TSHR* eQTLs near rs179252 and re-express the regional association rather than providing independent evidence. Cultured fibroblasts and adipose tissue are not orbital tissue. Direction, compared with the blood estimate in the same dataset. CI, confidence interval; eQTL, expression quantitative trait locus; GTEx, Genotype-Tissue Expression; OR, odds ratio; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.

Figure S1

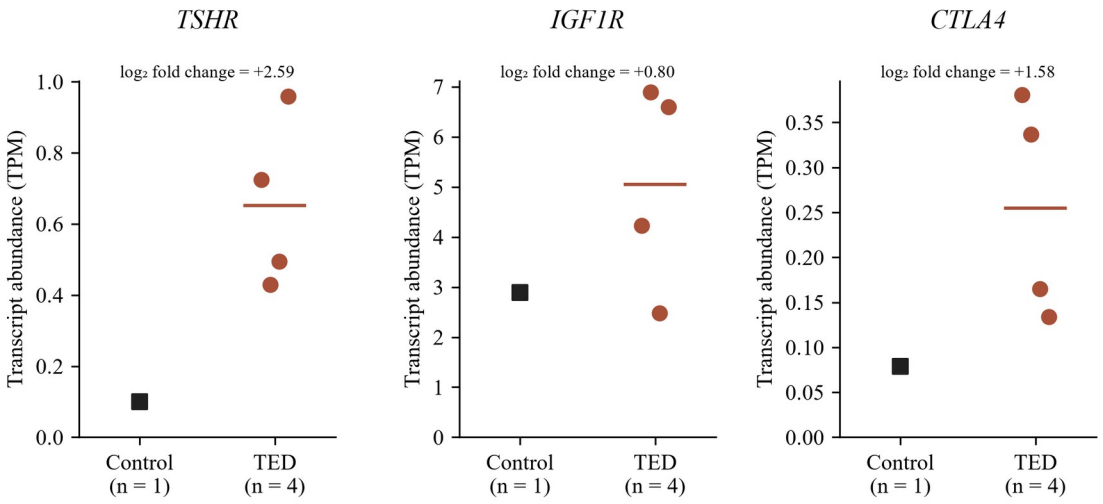


Figure S1. Descriptive orbital expression of *TSHR*, *IGF1R* and *CTLA4*. Transcript abundance in orbital adipose tissue collected during orbital decompression from four patients with inactive TED, and in nasal fat pad from one person without thyroid disease undergoing cosmetic upper blepharoplasty. Points are biological samples (technical replicates combined); horizontal lines show TED means. The $\log_2$ fold change is $\log_2$[(mean TED TPM + 0.01)/(control TPM + 0.01)]. Vertical axes differ by gene. The control came from a different fat depot, and one control cannot estimate control-group variance; no inferential test was performed, and these observations do not validate the genetic findings. TED, thyroid eye disease; TPM, transcripts per million.

Figure S2

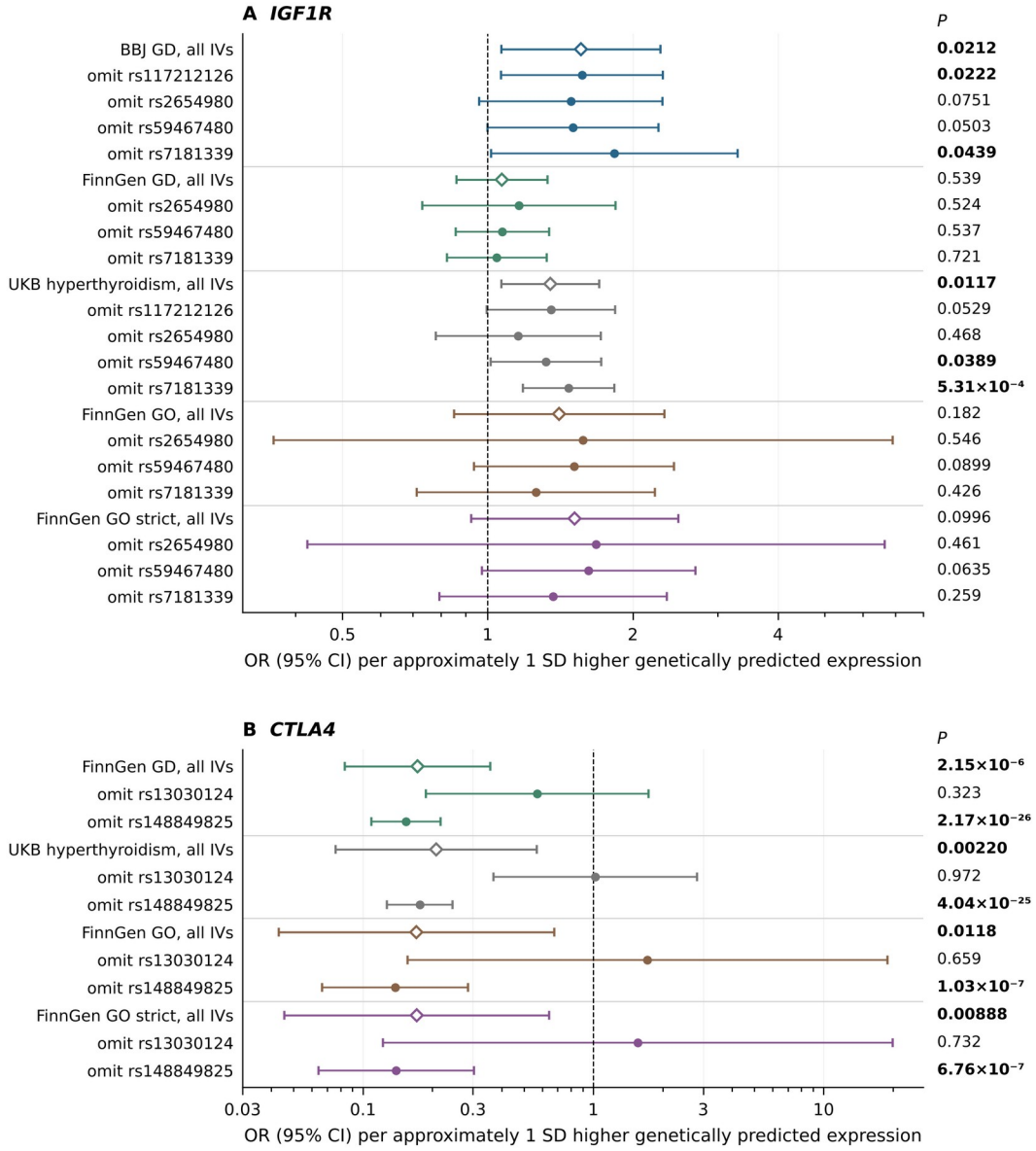


Figure S2. Dependence of *IGF1R* and *CTLA4* estimates on individual instruments. For each dataset, the open diamond is the estimate using all instruments (as in Table 2), and the filled points below it omit the named instrument (IVW when at least two instruments remain, Wald ratio when one remains). Lines are 95% CIs on a logarithmic axis; the dashed line is OR = 1. *P* values are shown at right; bold, nominal $P < 0.05$. (A) *IGF1R* in all five datasets. (B) *CTLA4* in the four FinnGen and UKB datasets; BBJ *CTLA4* and all *TSHR* estimates use one instrument and could not be assessed. The 25 post hoc omission estimates reuse the same association data and are not independent replications; loss of nominal significance can reflect reduced precision as well as the omitted variant's contribution. CI, confidence interval; IV, instrumental variable; IVW, inverse-variance weighted; OR, odds ratio; SD, standard deviation; BBJ, Biobank Japan; FinnGen GD, E4_GRAVES_STRICT; GD, Graves disease; GO, Graves ophthalmopathy; UKB, UK Biobank.